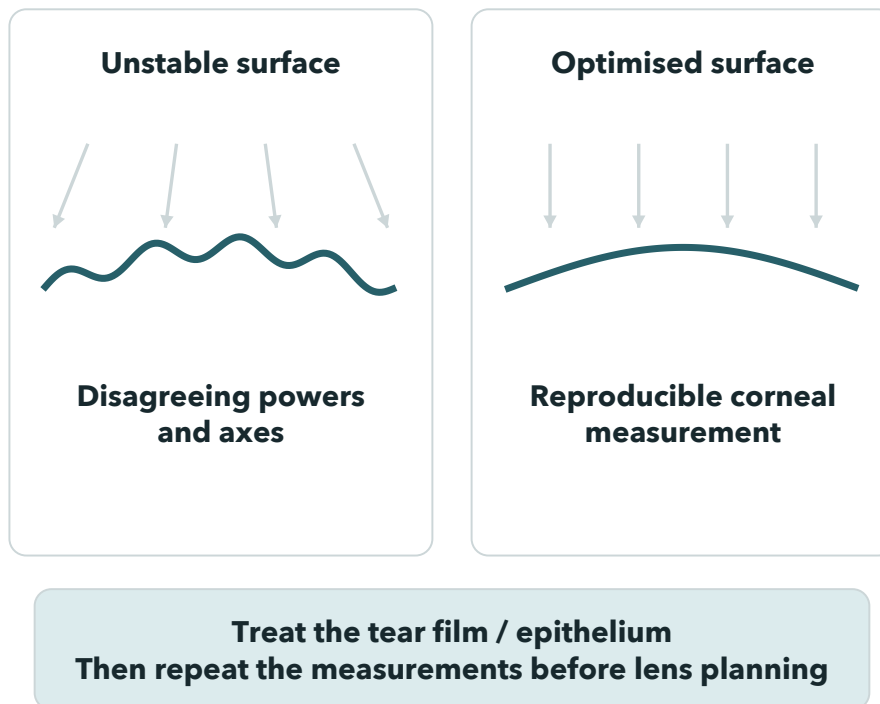


Cataract planning: measurements and risk clues

Optimise the surface, identify unstable support and coordinate active retinal disease.

A stable tear film makes a reliable measurement

A | Why repeated corneal powers disagree



Original tear-film and measurement schematic. Axial length can be consistent while tear-film-related keratometry remains unreliable.

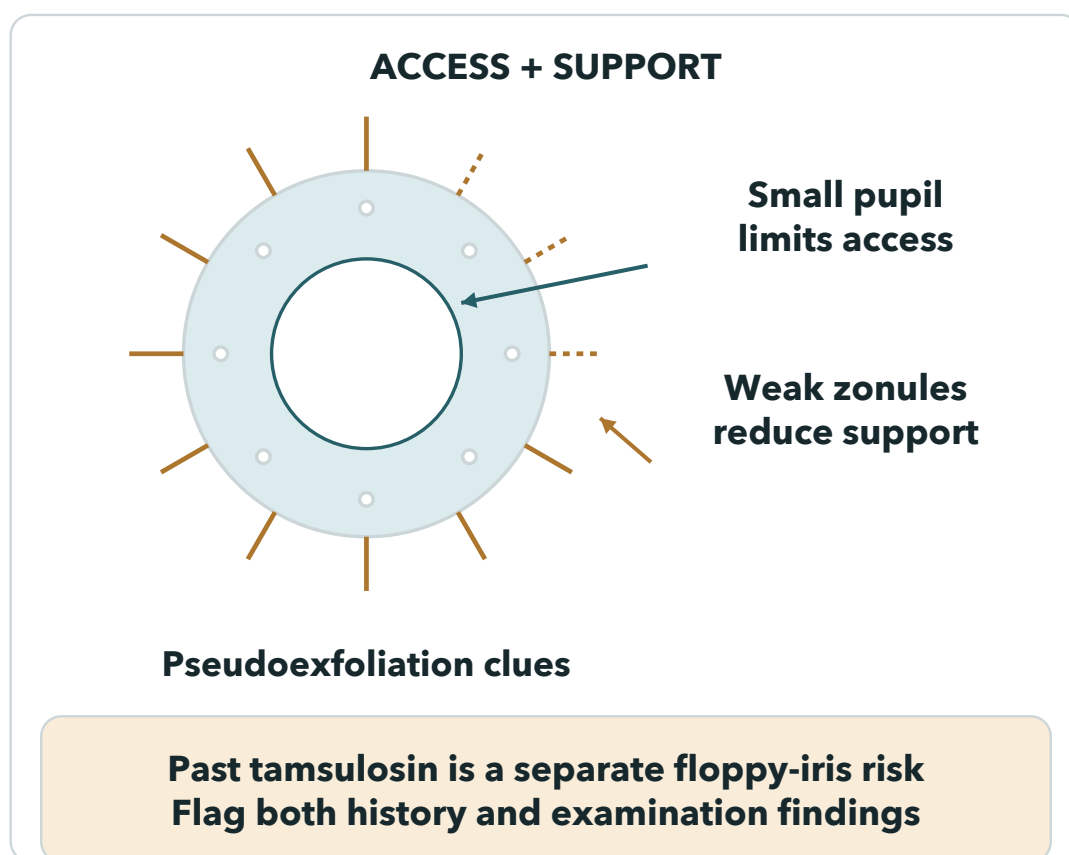
Before choosing the implant

Clue	Planning response
Fluctuating blur; staining	Optimise the ocular surface and repeat keratometry or topography.
Past tamsulosin	Flag floppy iris risk even after stopping; plan pupil management.
Flaky material; phacodonesis	Pseudoexfoliation suggests poor dilation and weak zonular support.
Active diabetic macular oedema	Coordinate retinal treatment, surgery timing and perioperative follow-up.

References ASCRS | Preoperative ocular surface algorithm | NICE NG77 | Cataract recommendations | NICE NG242 | Diabetes and cataract surgery

Anterior signs predict operative difficulty

B | Access and support are separate problems



Original anterior-segment schematic. Poor dilation limits access; weak zonules reduce capsular support. Tamsulosin and pseudoexfoliation are distinct risk clues.

Plan rather than assume

Document current and past relevant drugs, dilation and lens stability. These findings change preparation and counselling; they do not automatically make cataract surgery impossible. Coordinate drug decisions with the prescriber and do not assume that stopping an alpha-blocker removes risk. Establish the current diabetic eye status because the lens opacity and macular disease can both limit vision. Avoid finalising an intraocular lens from inconsistent surface measurements; assess the reason for variation first. One stable axial length result cannot rescue fluctuating corneal power measurements from a disrupted tear film. Likewise, the absence of tamsulosin from today's medication list does not erase a relevant past exposure. Review the history and the measurements before committing to the operative plan.

References AAO EyeWiki | Floppy iris and small pupils | AAO EyeWiki | Cataract in pseudoexfoliation | NICE NG242 | Diabetes and cataract surgery